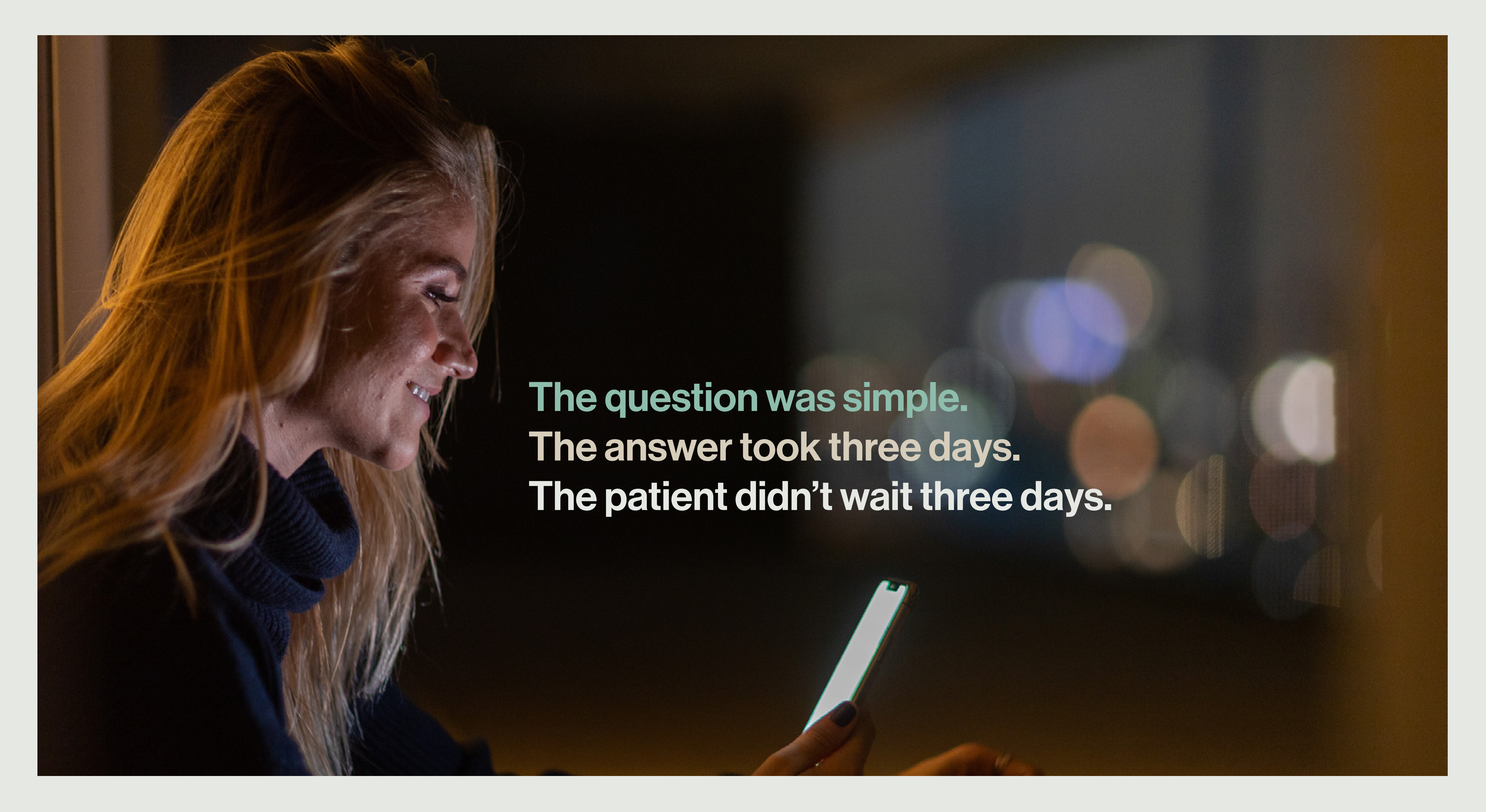


Every enquiry
you don't answer
costs you money.

AI-POWERED ELEVATED PATIENT COMMUNICATION FOR PRIVATE PLASTIC SURGEONS



**The question was simple.
The answer took three days.
The patient didn't wait three days.**

CARECONCIERGE

AI-POWERED ELEVATED PATIENT COMMUNICATION FOR PRIVATE PLASTIC SURGEONS

£2.7 billion plastic surgery market. One decisive advantage: who responds first.

£2.7B

UK cosmetic surgery clinic revenue — and climbing.

Growing at 7.3% CAGR. Social media and post-pandemic demand are accelerating the market. Almost all procedures are private pay by definition.

~300+

BAPRAS plastic surgeons in private practice

A credential-driven, reputation-led market. No dominant private-practice player. Most practices are run on goodwill, referrals, and a part-time secretary.

First.

Mover wins the consultation

The practice that responds first wins in the majority of elective surgery enquiries. Speed is the differentiator that GMC guidance and ASA rules can't restrict.

The competitive edge is no longer advertising.

ASA rules and GMC marketing guidance now tightly restrict what you can say and how you can say it. The edge belongs to whoever responds fastest, most intelligently, at any hour.

The advertising rulebook tightened in 2024. Most private practices are still marketing like it didn't.

What ASA/CAP and the GMC now restrict

- Testimonials in any advertising format, including social media posts
- Before-and-after imagery that is filtered, edited, or unrealistically framed
- Time-limited offers, discounts, or finance incentives used as inducements to book
- Exploitative language targeting body insecurities or post-pregnancy appearance
- Claims of being 'qualified' or 'experienced' without substantiated GMC Specialist Register evidence
- Ads placed where they are likely to reach under-18s without strict age-gating

GMC's updated December 2024 guidance: recognise patient vulnerability and market responsibly — active enforcement expected in 2026.

What this means for your practice

Ad spend is harder to justify.

When every testimonial, before-and-after, and urgency hook is restricted, awareness-only campaigns struggle to convert. You are paying for attention you can't act on.

Your first response is now your marketing.

The GMC has removed most of the tools that create consideration before the enquiry arrives. How fast you reply, how well it reads, whether it qualifies the patient — that is the differentiator.

CareConcierge is built for this environment.

Every response is built from your GMC-reviewed content. No testimonials. No restricted language. No exploitative framing. Compliant by design.

**You're competing with practitioners who have no such restrictions.
Your patients don't know the difference — yet.**

The landscape your patients are navigating

- Non-surgical aesthetics practitioners currently operate with minimal regulation in England.
- They can advertise freely with testimonials, before-and-afters, and promotional pricing.
- Media coverage of unsafe practice (liquid BBL deaths, Harley St scandals) has raised public anxiety — but also credibility demand for BAPRAS surgeons.
- New government licensing scheme (red/amber/green procedures) expected from 2026 — surgical procedures will remain under CQC/GMC.

Patients seeking a qualified surgeon are ready to commit — they just need to reach you first.

What this creates for you

A trust premium that is real.

BAPRAS membership, GMC registration, and CQC-registered premises already differentiate you from the unregulated market. Patients who know to look for this are your highest-converting leads.

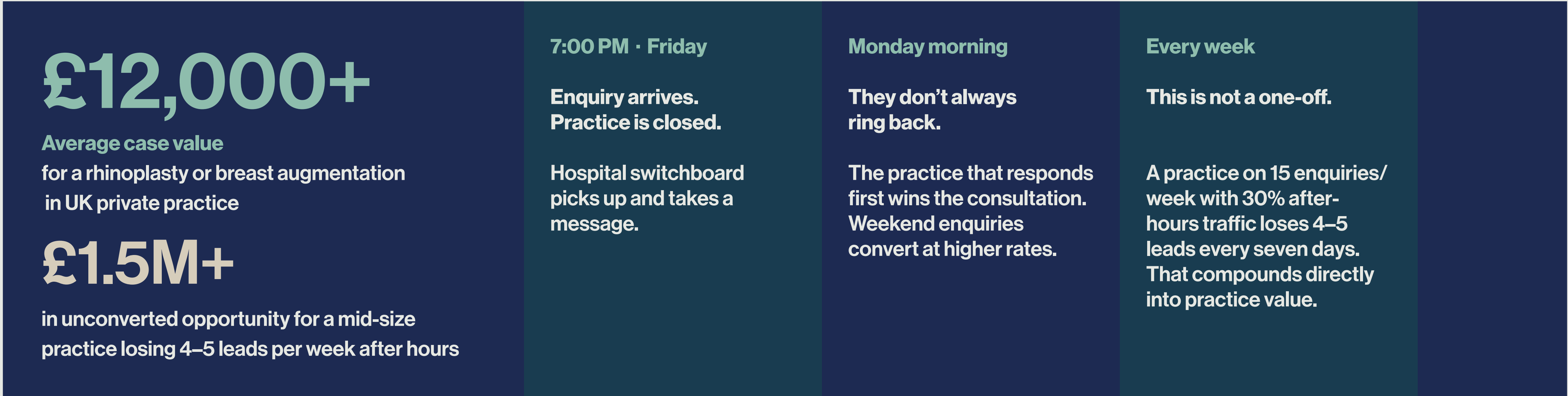
But you can't capture it if you don't respond.

The patient who finds you on a Sunday evening and doesn't hear back by Monday morning has already moved on — potentially to a less qualified provider who replied at 11pm.

Speed + credibility = practice growth.

CareConcierge gives you the response infrastructure to match your clinical reputation. No restricted content. No unqualified claims. Just fast, intelligent, compliant engagement.

The enquiry that arrives at 7pm Friday is your competitor’s patient by 9am Monday.



In UK private practice, the cost of missed enquiries isn't just a salary. It's often your own time.

What private practice admin actually costs

Base salary	£28,000–£38,000 / yr
Employer NI (13.8%)	+ £3,900–£5,200
Pension contribution (3%+)	+ £840–£1,140
Recruitment and onboarding	+ £2,000–£4,000

Total: approx.
£34,000–£48,000 per year.

And many private plastic surgeons in the UK don't have a dedicated coordinator at all. They rely on a Spire or Nuffield hospital secretary who splits time across multiple consultants — or they handle it themselves between theatre sessions.

Where their time actually goes

- ~40% Answering repetitive FAQ enquiries
Pricing, procedure info, recovery times. Answers that never change
- ~25% Chasing leads that went cold
Following up enquiries that arrived after hours
- ~20% Data entry and admin
Logging enquiries, updating records, scheduling.
- ~15% High-value patient conversations
The only work that genuinely requires a human

CareConcierge handles the 85%. Your coordinator — or you — focuses on the 15% that actually requires clinical context and human warmth.

Every enquiry answered. Every lead qualified. Every handoff ready on your desk. While you're in theatre.

01.
Enquiry Arrives

WhatsApp · Web chat SMS · Instagram DM Any hour. Any day.

02.
AI Responds

Immediate. Calibrated to your tone, scope, and GMC/ASA-compliant content. Feels considered, not automated.

03.
Lead Triage

Genuine enquiries separated from noise before any staff member is involved. The AI qualifies.

04.
Human Handoff

At the decision-ready moment, your coordinator receives a warm, detailed handoff — not a cold form.

05.
Treatment Booked

Revenue that would have gone cold is protected. The system works while you operate.

24/7/365

after-hours coverage

GMC/ACA-aware

no restricted language, ever

Purpose-built

for private surgical practices

From £1,450

per month



“We had considered hiring weekend staff purely to cover the after-hours enquiry volume. CareConcierge solved that problem entirely.”

Dr Pouria Moradi, ASPS Specialist Plastic Surgeon, Sydney

Built and proven in surgical private practice in Sydney, Australia. Here's what happened.

What changed at the practice

Staff freed from FAQ admin

The team was spending significant time on repetitive enquiries about pricing, procedures, and availability. That workload now routes to AI before any human is involved.

After-hours leads captured — not lost

Every enquiry is now acknowledged immediately, qualified, and held for the coordinator with full context when they return to their desk.

Patients responded positively

Multiple patients commented on the quality and speed of the response — without knowing it wasn't a human. The impression of the practice was elevated before they ever met the team.

Investment: £1,450/month

Full platform. Every channel. 24/7 coverage. Monthly Conversion Intelligence Report. Setup on a 12-month commitment.

£1,450

Per month. All channels. Full setup.
Conversion Intelligence Report included.



Dr Pouria Moradi
Specialist Plastic Surgeon · Sydney

CareConcierge co-founder
and founding case study

One recovered surgical case per month covers the platform for the year. Most practices see that in the first twelve days.

	SETUP	MONTHLY	TERM	WHAT YOU GET
Practice Solo practice — entry point	£1,750	£725 / month (anchor tier)	12 months	Full setup, all channels, out-of-hours coverage, 30-day review, up to 400 AI conversations/month
Growth (Recommended) The primary offer	£2,750	£1,450 / month	12 months	Everything in Practice, monthly Conversion Intelligence Report, lead nurture sequences, quarterly strategy session, up to 800 AI conversations/month
Elite High-volume or multi-site	From £3,750	From £2,200 / month	Custom	Everything in Growth, dedicated account manager, custom AI training per procedure, unlimited AI conversations

You're live in three weeks. Nothing changes for your team — except the enquiries stop going cold.

01. CRAWL

Weeks 1–3

30-minute intake interview.

We run a structured Zoom call with your team. We ask the questions — you don't need to prepare a brief. From that conversation we build your AI knowledge base: services, pricing parameters, tone of voice, escalation rules, and every FAQ your team currently answers by hand.

No changes to your PMS, your CRM, or your reception workflow.

02. WALK

Months 1–3

Live system. Real data.

The AI handles enquiries across your configured channels. Your team receives only vetted, qualified leads. We run a 30-day review together: what's working, what needs refining, what the data shows.

Deliberately intentional. We learn how your practice actually runs before proposing anything more.

03. RUN

Month 3 onwards

Managed optimisation.

Monthly performance review and script refinement. Conversion tracking from enquiry through to consultation and treatment start. Optional expansion to additional channels. Lapsed patient reactivation when you are ready.

“Isn’t this just a chatbot?” No. Here’s the distinction worth understanding.

It’s the first question every surgeon asks. And it’s the right one.

A chatbot

A decision tree.

Scripted paths. Scripted answers. If a patient asks something outside the designed route, they hit a wall: “I’m sorry, I don’t understand your question.” The conversation ends. The lead is gone.

Optimised to deflect.

Feels like a FAQ page that talks.

CareConcierge

A conversational agent.

It has one goal: booking a consultation. Everything else serves that. If a patient goes off-script, it answers and steers them forward. The conversation never hits a wall.

Optimised to convert.

Feels like your best coordinator.
At 11pm on a Sunday.

The guardrails are clinical, not conversational.

Every response is built from your GMC-reviewed practice content. CareConcierge will never speculate about surgical outcomes, quote fees out of context, or use restricted language. The intelligence is real. The compliance is absolute.

Your most valuable patients already know you. They just went quiet.

Every practice has them. Patients who enquired, requested a consultation, showed genuine intent — and then went silent. Not because they chose someone else. Because life got in the way, the timing wasn't right, or nobody followed up before the moment passed. They are sitting in your system right now. Warm, qualified, and waiting to be asked again.

What CareConcierge does with them

A structured reactivation sequence runs automatically to your lapsed patient list — personalised, compliant, calibrated to your tone. Not a bulk email blast. A considered conversation that picks up where you left off.

“We haven't heard from you in a while. Your consultation is still available whenever you're ready. Here's how to get back in touch.”

These patients have already chosen you. They've done the research. The barrier to re-engagement is low — but only if someone reaches out. CareConcierge does it automatically, at scale, without adding a single task to your team.

The maths are disproportionate.

A mid-size practice with 200 lapsed enquiries in the last 18 months, converting at 12% through reactivation, recovers 24 consultations.

At £12,000+ average case value, that's £288,000 in protected revenue — already sitting in your system. Untouched.

CareConcierge finds it. While you're in theatre.

We are taking ten practices this quarter. That is the entire first United Kingdom cohort.

Direct access to the founding team

Greg, Oli, and Pouria — not a support queue. You have the principals.

Conversion Intelligence included from month one

Every month you receive a full report: enquiries handled, consultations generated, estimated revenue protected.

Your practice shapes the product

Feedback from the first ten directly informs the platform roadmap.
You are building this with us.

A credentialed co-founder as your peer reference

Dr Pouria Moradi is available for direct, peer-to-peer conversations.
No sales pitch. Surgeon to surgeon.

Schedule your consultation

A 30-minute conversation. No obligation, no pitch deck to sit through. We ask about your current enquiry flow, explain exactly what the system does and doesn't do, and you decide.

BOOK A TIME

Gregory Gray

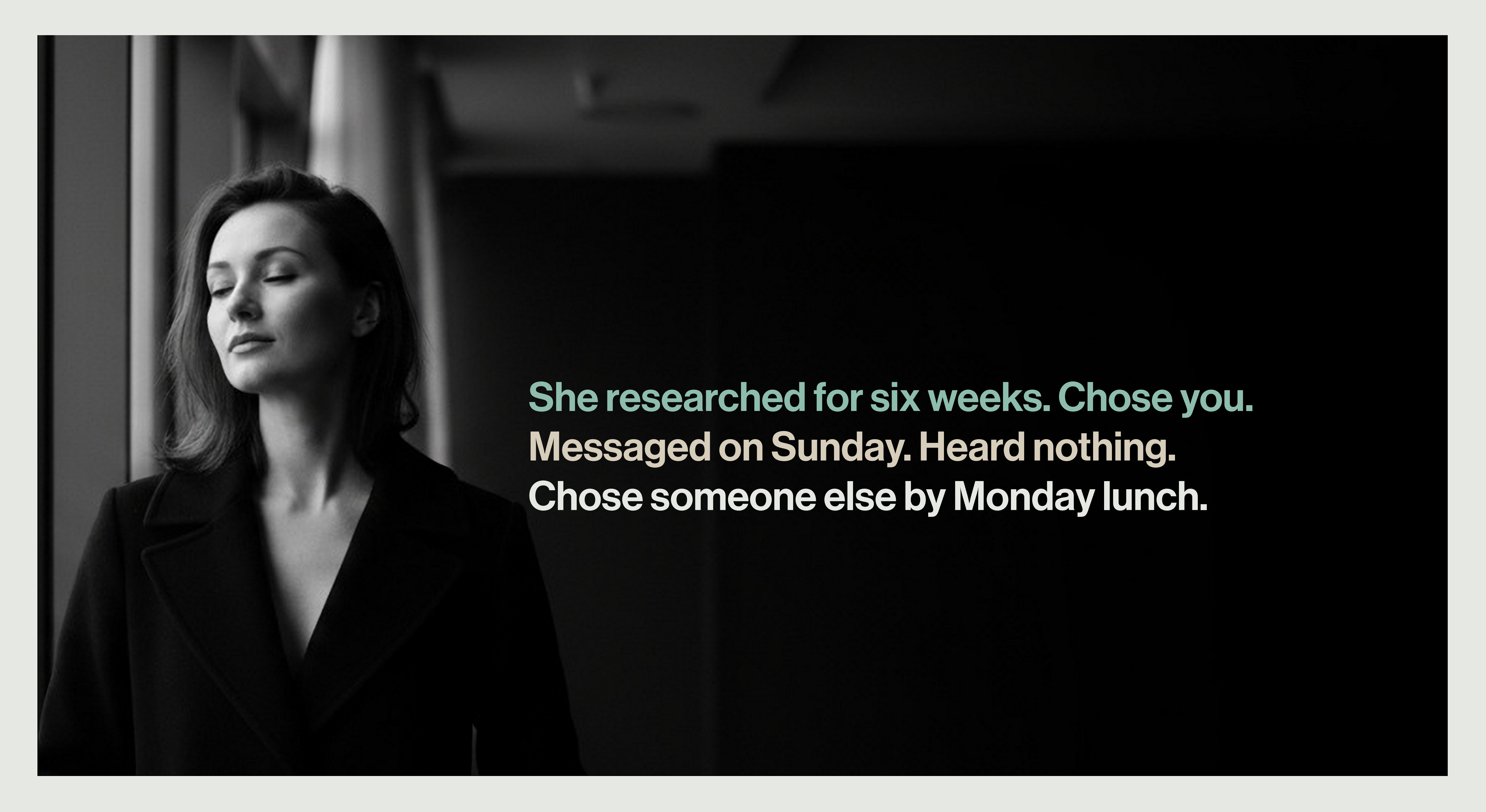
Director & Co-Founder,
CareConcierge Pty Limited

Mobile. +61 430 388 662

Email. greg@careconcierge.health

Schedule a Meeting

<https://calendly.com/greg-careconcierge/30min>



**She researched for six weeks. Chose you.
Messaged on Sunday. Heard nothing.
Chose someone else by Monday lunch.**

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